

# Urine - Blood In - Male (Child)

Algorithm 1479 | Child (0-17 years) | Gender at birth: Male

## Definition

- UAT-only adult and pediatric assessment of reported visible blood in urine, separating life-threatening bleeding, retention/obstruction, infection, trauma and pregnancy-related concerns from stable haematuria that still requires in-person testing.
- Child patient.
- Male.

## Initial assessment

1. [OPEN\_TEXT] Is blood definitely mixed with urine, and is it pink, red or dark brown, streaked, clotted, or causing heavy bleeding? Could it instead be vaginal/menstrual, rectal, skin or genital bleeding?
2. [DURATION] When was it first and last seen, how many times, and is it continuing or increasing?
3. [YES\_NO] Is there inability to urinate, only drops despite a full painful bladder, clots, severe side/back/abdominal/groin pain, fever or rigors, vomiting, reduced urine, swelling, confusion, faintness, pallor, or serious illness?
4. [YES\_NO] Was there recent abdominal, back, pelvic or genital trauma, a procedure or catheter problem, strenuous exercise, or possible foreign body?
5. [OPEN\_TEXT] What is the patient's age and anatomy, and are they pregnant or recently postpartum, taking anticoagulant/antiplatelet medicines, or known to have kidney, urinary, bleeding, immune or cancer conditions?
6. [YES\_NO] For a child or adolescent, is there fever, abdominal mass/swelling, bruising, genital injury/discharge, reduced urine, marked distress, or any privacy or safeguarding concern?
7. [CAREGIVER\_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

## Triage and redirect questions

### **1. Redirect → Flank Pain**

Is pain in the side or back the main concern rather than visible blood in urine?

Pain focused in the side or back belongs in the Flank Pain pathway.

### **1. Disposition level 100**

Is there heavy or ongoing bleeding, clots with inability to urinate or painful bladder distension, fainting/shock, severe illness or confusion, severe trauma, severe flank/abdominal pain with vomiting or reduced urine, or an unwell infant or child?

Heavy bleeding, clot retention, shock, serious infection, obstruction and significant urinary-tract trauma require emergency assessment. A child may deteriorate without being able to describe symptoms reliably.

### **1. Disposition level 70**

With no emergency feature, is there any possible visible blood in urine, including a first, single, painless or small episode, or blood with dysuria, fever, flank pain, pregnancy/postpartum status, anticoagulant use, recent trauma, or childhood?

NHS.UK advises prompt assessment of any visible haematuria. Urine testing and sometimes blood tests, examination or imaging are needed; food, medicines or menstruation must not be assumed to explain it remotely. Unexplained visible haematuria in a child warrants very urgent specialist assessment under NICE suspected-cancer guidance.

## Advice

### ***Qatar urinary-bleeding emergency response***

- Call Qatar emergency services on 999. Do not allow self-driving. Do not force fluids when urine cannot pass, the bladder is painfully full, vomiting is ongoing, or emergency assessment is required. Do not insert, flush or manipulate a catheter unless specifically trained and directed. Report prescribed anticoagulant/antiplatelet medicines, but do not stop them unless the emergency clinician directs it. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: increasing bleeding or clots; cannot urinate; fainting or confusion; worsening pain; reduced responsiveness.

### ***Prompt in-person haematuria assessment***

- Arrange prompt in-person assessment through an approved Qatar adult or pediatric pathway for urine testing and clinician-directed examination, blood tests or imaging. Do not assume food, menstruation or medicine is the cause and do not start leftover antibiotics. Children with unexplained visible blood need a very urgent age-appropriate pathway. Call 999 for heavy bleeding, clots with retention, severe pain/illness, fainting, confusion or deterioration. The exact Qatar non-emergency urology, nephrology, pediatric, maternity and trauma destinations are GOVERNANCE\_REQUIRED.
- Call Back If: heavy bleeding or clots; cannot urinate; fever or rigors; flank pain or vomiting; reduced urine; child becomes unwell.

## First aid

- Call Qatar emergency services on 999. Do not allow self-driving. Do not force fluids when urine cannot pass, the bladder is painfully full, vomiting is ongoing, or emergency assessment is required. Do not insert, flush or manipulate a catheter unless specifically trained and directed. Report prescribed anticoagulant/antiplatelet medicines, but do not stop them unless the emergency clinician directs it. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

## References

- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (accessed 2026-07-25)
- NHS.UK, "Blood in urine", <https://www.nhs.uk/symptoms/blood-in-urine/> (accessed 2026-07-25)

- NICE NG12, "Suspected cancer: recognition and referral", recommendations for visible haematuria in adults and children (updated 2026)

## Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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